A/PROF CHAMARA BASNAYAKE

MBBS, PhD, FRACP

Gastroenterologist & Endoscopist

PN 421926PK

FOCUS GASTROENTEROLOGY

01 May 2026

Dr Glen Guerra
Melbourne Colorectal Specialists

Dear Dr Guerra,

Re: Trajan Geogievski

RE: Mr Trajan Georgievski DOB: 13/09/1943 Ph: / 0423765693

58 Harman Avenue ST ALBANS VIC 3021 3021 92621 9 1

olga.georgievski@y7mail.com (daughter)

Dear Glen I would be grateful if you could see Trajan for the management of a 9cm palpable non-incarcerated inguinal hernia (LEFT) where it appears colon is involved, on a background of diverticulosis, new lower bowel symptoms and upper GI symptoms. 30 years ago had a right sided inguinal hernia repair. No major comorbidities.

My letter to GP below

Chamara

Summary

- **• Recurrent bolus dysphagia likely secondary to reflux-related oesophagitis and a 4cm hiatus hernia.**
- **• Large 9cm inguinal hernia potentially contributing to lower abdominal symptoms**
- **• Optimisation of proton pump inhibitor (PPI) therapy and referral for surgical consultation for inguinal hernia.**
- **• No action required by you.**

I had the pleasure of conducting an in-person consultation with Trajan Georgievski with his daughter Olga regarding recurrent dysphagia and chronic constipation.

Symptoms and history of presenting complaint

Mr Georgievski describes a 12-month history of progressive dysphagia, primarily localised to the mid-to-lower retrosternal region. Symptoms are most pronounced with solids, specifically bread and meat. A few weeks ago, he experienced a significant bolus obstruction lasting eight to nine hours, associated with hypersalivation and an inability to clear secretions, which required an emergency department presentation. While liquid intake is generally preserved, he has developed significant food phobia and has restricted his diet accordingly. He has been trialled on pantoprazole but reports associated dizziness and has been taking the medication suboptimally after food.

All appointments and enquires

Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
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Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations

Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Additionally, he reports a change in bowel habit characterised by chronic constipation following a previous course of antibiotics for *H. pylori* eradication. He describes a persistent sensation of abdominal heaviness and tenesmus, often requiring up to eight attempts daily to achieve incomplete evacuation. He denies haematochezia or melaena.

Diet

Mr Georgievski follows a Mediterranean-style diet with high bread and carbohydrate intake. He has recently reduced his meat consumption due to swallowing concerns and has remained abstinent from alcohol for several months. His caffeine intake is moderate, consisting of one Turkish coffee and several teas daily.

Past medical history and medications

His medical history is significant for a 4cm hiatus hernia previously identified on gastroscopy, a benign brain tumour which is managed conservatively with serial imaging, and a right-hand tendon repair performed 35 years ago. His current medications include telmisartan for hypertension, atorvastatin for hypercholesterolaemia, metoprolol, and pantoprazole 40mg. For his bowels, he uses Coloxyl as required.

Social history, family history

He is a retired forklift supervisor and lives with his wife. There is no reported family history of gastrointestinal malignancy, inflammatory bowel disease, or coeliac disease.

Examination

On physical examination, his abdomen was soft and non-tender. There was a palpable, sizeable hernia in the left abdominal/inguinal region, which was prominent upon coughing. This correlates with his recent CT imaging. I also reviewed previous gastroscopy reports which confirmed a 4cm hiatus hernia and histological evidence of reflux-related oesophageal changes.

Impression and plan

The primary clinical concern is recurrent bolus dysphagia. This is most likely secondary to gastro-oesophageal reflux disease (GORD) exacerbated by his hiatus hernia, possibly involving a peptic stricture or associated dysmotility. His current medication regime has been hampered by side effects and incorrect timing. To address this, I have advised him to switch to an alternative PPI brand to mitigate dizziness and emphasised that this must be taken 30 minutes prior to breakfast and dinner to ensure maximal efficacy. Its worth noting he has felt symptomatically better from a dysphagia perspective since being on a PPI.

We discussed the option of an urgent gastroscopy for balloon dilatation of any potential stricture; however, as his symptoms have partially improved, Mr Georgievski prefers a conservative trial of high-dose PPI for two months. If he experiences any further bolus obstruction, I have advised him that an endoscopic intervention will be mandatory. As an emergency measure for food impaction, I have suggested he trial a small amount of carbonated beverage (Coca-Cola) to facilitate bolus passage.

Regarding his constipation, the CT scan from December identifies a 9cm hernia containing segments of the colon. I suspect this mechanical factor is a significant contributor to his difficulty with evacuation. I have recommended simple measures including the daily consumption of kiwi fruit and the use of a footstool to

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optimise the anorectal angle during defecation.

I will refer him to a general surgeon at St Vincent's for an assessment of the 9cm hernia, as surgical repair may be necessary to resolve his bowel symptoms. I will review him in three months, via telehealth if preferred, to monitor his progress.

Thank you again for referring this patient.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

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